

--SUMMARY--

Decision No. 1563/12

22-Feb-2013

R.McCutcheon

- Second Injury and Enhancement Fund {SIEF} (severity of accident)
- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)

The worker suffered a knee injury in 2008. The employer appealed a decision of the Appeals Resolution granting the employer only 50% SIEF relief. The employer submitted that it should be entitled to 90% SIEF relief.

The worker slipped and fell directly on her knee on a hard surface. She required assistance to stand up and immediately went to have the knee iced. At the emergency room the next day, she characterized her pain as 8/10. Percocet was prescribed, and x-rays were taken to rule out a fracture. Considering the facts and Tribunal case law, the Vice-Chair concluded that the severity of the accident was moderate.

Medical documentation referred to extensive degeneration of the knee. The Vice-Chair found that the pre-existing degeneration played a major role in prolonging the claim and in the need for surgery. The medical significance of the pre-existing condition was major.

Based on an accident of moderate severity and a pre-existing condition of major medical significance, the employer was entitled to 75% SIEF relief. The appeal was allowed in part.

9 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w1513s]
- TRIBUNAL DECISIONS CONSIDERED: 1174/06 consd, 529/07 consd, 1582/07 consd, 2290/09 consd, 799/10 consd

Style of Cause:

Neutral Citation: 2013 ONWSIAT 407



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1563/12

BEFORE:

R. McCutcheon: Vice-Chair

HEARING:

August 14, 2012 at Toronto
Written

DATE OF DECISION:

February 22, 2013

NEUTRAL CITATION:

2013 ONWSIAT 407

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) K. Hille decision dated August 4, 2011

APPEARANCES:

For the worker:

Not participating

For the employer:

Mr. M. Mitchell, Paralegal

Interpreter:

Not applicable

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

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REASONS**(i) Introduction to the appeal proceedings and the issue**

[1] The employer appeals a decision of the ARO, which concluded that the employer was not entitled to an increase in the 50% cost relief under the Second Injury and Enhancement Fund (SIEF) granted for this claim. The ARO rendered a decision based upon the written record without an oral hearing.

[2] At the Tribunal, this appeal was considered on the basis of written submissions. The employer's representative filed a written submission dated January 27, 2012. The employer seeks 90% SIEF relief in respect of the costs of this worker's claim.

(ii) Background

[3] The following facts give rise to this appeal:

- The worker was employed as a hospitality service aid and was hired by the accident employer in 1982. Her job duties involved cleaning in a hospital.
- The accident at issue occurred on November 2, 2008. The worker was walking to her vehicle after her shift, when she turned her ankle and fell on her right knee. The worker was 52 years of age at the time of the accident.
- The worker was treated at the emergency department the next day, with complaints of knee and low back pain. The diagnosis was contusion right knee, post-traumatic right patello-femoral arthritis, and right sacroiliac strain.
- The worker followed up at a medical clinic on November 4, 2008. The worker was unable to weight-bear and the range of motion of her right knee in flexion was decreased by 50%. The worker was prescribed a Zimmer splint, Oxycocet, and Arthrotec. At that time, it was thought that the worker could return to work with restrictions against squatting and kneeling, and standing longer than 10 minutes. The worker was directed to wear the Zimmer splint continuously when sitting or standing.
- The worker then saw her family doctor, Dr. Hyatt. Dr. Hyatt referred the worker for physiotherapy and an MRI scan. Dr. Hyatt recommended that the worker rest her knee and not return to work.
- After making inquiries, the WSIB adjudicator concluded that the worker sustained an injury arising out of and in the course of her employment and granted initial entitlement. The adjudicator also accepted that the worker's time off from work was medically authorized and granted loss of earnings (LOE) benefits.
- The worker's recovery was somewhat slower than expected. The worker's age was cited as a potential factor delaying recovery. In March 2009, the WSIB granted a physiotherapy treatment extension request.
- The worker also received steroid injections which provided some temporary relief.
- The worker was referred to Dr. Martin for a specialist consultation. The worker underwent arthroscopic surgery on October 22, 2009. The operative report provided the following

post-operative diagnoses for the right knee: chondral degeneration of all three compartments; medial meniscal tear; distal patellar tendinopathy.

- In March 2010, the worker returned to part-time modified work with the accident employer, working 25 hours per week. She had medical restrictions against climbing stairs, bending, mopping floors, and heavy lifting.
- The WSIB referred the worker for a multi-disciplinary assessment, which took place in May 2010. That assessment concluded that the worker's restrictions were permanent.
- The worker was referred to Dr. Cameron, orthopaedic surgeon. Dr. Cameron recommended a total knee replacement, which took place on October 1, 2010. The diagnosis was osteoarthritis of the right knee.
- In May 2011, the accident employer offered modified duties to the worker in her original job as hospitality aide. It appears that the worker accepted this offer and returned to modified work.
- In June 2010, the employer requested SIEF cost relief. The WSIB determined that the employer was entitled to 50% cost relief on the basis of a moderate accident history and a moderate pre-existing condition.
- The ARO upheld the decision to grant 50% SIEF cost relief and the employer appeals to the Tribunal.

(iii) Law and policy

[4] Since the worker was injured in 2008, the *Workplace Safety and Insurance Act, 1997* (the "WSIA") is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[5] Document No. 14-05-03 of the *Operational Policy Manual (OPM)*, "Second Injury and Enhancement Fund," governs this appeal and states in part as follows:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

...

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

...

Application to Employer Costs

Different factors may apply in recommending relief for the employer as compared to what is appropriate for determining a worker's benefit. In determining the amount of financial relief given to an employer, consideration is given to the medical significance of the pre-existing condition and the severity of the accident.

Cases of Permanent Impairment

If the transfer of costs to the SIEF exceeds 50% the employer receives the benefit applied to all or part of the claim, depending on the individual circumstances.

[6]

The policy sets out the application of SIEF to employer costs in the following chart and explanatory notes:

SIEF-Application to Employer Costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

Notes

*The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

**The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

Definition "Severity of Accident"

Minor: expected to cause non-disabling or minor disabling injury

Moderate: expected to cause disabling injury

Major: expected to cause serious disability probable permanent disability

***The percentage of the total cost of the claim transferred to the SIEF.

[7]

The policy cites section 98 of the WSIA as statutory authority for the SIEF policy. Section 98 states:

98(1) The Board may establish a special reserve fund to meet losses that may arise from a disaster or other circumstance that, in the opinion of the Board, would unfairly burden the employers in any class.

(2) Subsections 97(3) to (8) apply with necessary modifications with respect to the special reserve fund.

(iv) Conclusions and analysis

[8] The appeal is allowed in part. For the reasons that follow, I find that the employer is entitled to 75% SIEF cost relief because the accident was moderate and the worker had a major pre-existing condition.

(a) Severity of accident

[9] I find the Vice-Chair's reasoning in *Decision No. 529/07* to be instructive in interpreting the severity of accident:

This appeal raises the question of what is meant by the use of the word "expected" in the Board's SIEF policy, especially in the context of a disablement claim. In the case of a disablement, the worker often suffers a gradual onset of injury while doing his or her regular job. The question is whether "expected" in this context means "likely" or "probable" in the usual sense. If injury were probable, the work would be unsafe *per se* and likely outside regulatory requirements. As the employer representative submits, in most cases of gradual onset injury, the work results in only a possibility of injury. Therefore, if "expected" means "probable", then, on this interpretation, generally, a gradual onset injury would almost always be considered "unexpected", and the result of a "minor" accident. However the SIEF policy does not stipulate that all disablements should be found to be the results of minor accidents. I consider it likely that the Board would have specified this, if that had been the intent of this wording.

I note that in *Decision No. 877/01*, the Vice-Chair found that a disabling injury was not "expected" because if a work activity was expected to cause disabling injury, this would raise health and safety concerns. However, in *Decision No. 567/97*, a Vice-Chair found that a disablement was the result of a moderate accident. The Vice-Chair found that accidents leading to disablements might be assessed as being of different severity depending on the length of time required to trigger an overuse or repetitive strain injury.

In addressing this issue, in a disablement claim, I do not consider it reasonable to interpret the word "expected" to mean that injury is probable. I interpret it to mean only that an injury of the type experienced is reasonably possible, or not unexpected. The worker worked for seven years as a sewing machine operator, with ongoing repetitive movements of her shoulders. I do not find it unexpected that she suffered tendonitis or even a rotator cuff tear. While permanent impairment might not be expected, this work is recognized as involving risks for these types of injuries. The definition of an accident of moderate severity is of one expected to cause disabling injury but not necessarily permanent disability. I agree with the ARO that the accident was of moderate severity.

[10] Board policy defines the severity of an accident with reference to the extent to which it is expected to be "disabling." The Tribunal has generally interpreted the term "disability" to connote a loss of earning capacity.¹ Therefore, I interpret the term "disabling injury" to refer to the extent to which an accident would be expected to render the worker unable to work or perform other tasks of daily living. According to Board policy, a minor accident is expected to cause non-disabling or minor disabling injury. I construe this to mean that a worker involved in a minor accident would be expected to return to work immediately, or at most, a day or two later.

¹ See for example, *Decision Nos. 1070/94 and 518/96*.

[11] In my view, a moderate accident is one that would reasonably be expected to cause some disability in the form of several days or weeks off from work, or a period of modified work. The policy does not define the length of the expected disability associated with a moderate accident, although it is implied in the definitions of “moderate” and “major” that a moderate accident is not expected to cause permanent disability. This approach is consistent with the interpretation adopted in *Decision No. 529/07*.

[12] In determining the question of the severity of the accident, I have also considered the Tribunal decisions addressing similar circumstances. In *Decision No. 1174/06*, the Vice-Chair concluded that the worker’s fall represented a moderate accident, reasoning in part:

The accident in which the worker sustained her injury occurred when the worker tripped on a fork of a fork-lift truck and fell forward landing on her right arm. The force of the fall caused her to fracture her right wrist.

[...]

With respect to the accident itself, Mr. Roberts has submitted that the worker’s accident was minor. However, in my view, the accident was a moderate one under the Board’s policy, which defines such an accident as one that would be expected to “cause disabling injury”. I am persuaded that the fall of the kind experienced by the worker could reasonably lead to a fracture of the wrist. I am also persuaded that a fracture of the wrist would be expected to cause disabling injury, albeit likely only temporary. Consequently, I find that the severity of the accident was “moderate” under the Board’s policy.

[13] *Decision No. 2290/09* is another example of a decision addressing a slip and fall. The Panel in that case concluded that the accident was moderate in severity:

In our view, the severity of the accident is properly categorized as moderate. The worker tripped over an object. His fall was broken by his right hand hitting the concrete floor, with his right arm taking the force of the landing. In our view, an event involving force of this nature would be expected to cause a disabling injury. That is the definition of a moderately severe accident, as set out in the policy.

[14] *Decision No. 799/10* addresses a different type of accident, but I find that the Vice -Chair’s reasoning provides insight into the distinction between a minor and moderate accident:

In my view, the lifting of a heavy weight was one significant contributing factor to the worker’s disc herniation. This incident was not simply a minor event. A minor accident after all is expected to cause non-disabling or a minor disabling injury. Lifting a heavy weight, in my view, could, particularly if someone is ill-prepared for that, be expected to cause some degree of injury. In my view, the accident is best described as being “moderate” in severity.

[15] In support of his argument that the worker’s accident was minor, the employer’s representative cited *Decision No. 2505/09*, which applied *Decision No. 1238/07*. In *Decision No. 1238/07*, the vice-chair found that a fall backwards from a height of three to four feet was a minor injury. For reasons I have previously explained,² I prefer the approach taken in the decisions discussed above to the reasoning in *Decision No. 1238/07*.

[16] In addition, in *Decision No. 2403/12*, Vice-Chair Smith noted that a slip and fall accident is a recognized cause of disabling injury. I agree with this observation and find that this general

² See, for example, *Decision No. 1404/11*.

proposition is more in keeping with the generally expected consequences of a slip and fall on a hard surface. See also *Decision No. 1917/05*.

[17] The severity of accident is defined by the generally expected consequences, not the actual outcome. The employer's representative has not referred to any scientific or statistical evidence regarding the expected outcome of a fall on a hard surface.

[18] In the absence of direct medical evidence, the worker's initial treatment following the accident is relevant in determining the severity of the accident. In this case, the worker slipped and fell directly on her right knee on a hard surface. According to a Form 8 dated November 20, 2008, the worker required assistance from a passerby to stand up from the fall and she immediately went to have the knee iced and rested. She could not stand on it in the morning. The worker went to the emergency room the next day. The case was characterized as "less urgent", rather than non urgent. The worker rated her pain 8/10 and Percocet was prescribed. The worker's accident was considered significant enough to refer her for an x-ray to rule out a fracture. All of these factors point to an accident that was moderate, rather than minor.

[19] Taking into account Tribunal case law and the facts of this case, I find that the accident was moderate in nature.

(b) Significance of pre-existing condition

[20] Entitlement to SIEF requires the presence of a pre-existing condition that prolonged or enhanced the disability in the claim. In determining the significance of the pre-existing condition, I agree with the approach taken in *Decision No. 1582/07*:

I interpret the policy to mean that the medical significance of a pre-existing condition should be considered to be "minor" if it made the worker slightly more liable to develop a disability of greater severity than a normal person, and that it should be considered "major" if it made the worker extremely liable to develop a disability of greater severity than a normal person. If the extent to which the pre-existing condition made the worker more liable to develop a disability of greater severity than a normal person was more than slight, but less than extreme, the medical significance of the pre-existing condition could be considered moderate.

[21] In this case, I note the following evidence regarding the worker's pre-existing condition:

- Dr. Martin's operative report for the arthroscopy in October 2009 describes advanced degeneration in the right knee:

Inspection of the patellofemoral compartment revealed severe grade 3 generalized damage on both sides of the joint which was debrided with the powered shaver and smoothed out with the RF wand. ...

The medial compartment showed generalized grade 3 chondral damage of most of the weight bearing portion of the medial femoral condyle. ...

There was minor grade 2 wear on the anterior half of the tibial surface which was debrided...then there was some obvious degenerative tearing of the inner third of the posterior horn of the medial meniscus which was resected with the basket punch leaving stable tissue behind. The cruciate ligaments were intact.

Inspection of the lateral compartment showed that the meniscus was intact. The femoral surface was normal. There was significant grade 3 chondral damage in the very central

portion of the lateral tibial surface, narrow anteriorly but broad posteriorly where it disappeared underneath the posterior horn of the lateral meniscus. ...

- Dr. Martin noted in January 2010 that the worker did have “significant chondral degeneration throughout her knee, but she should settle down satisfactorily.”
- The MRI report of June 2010 offered the following interpretation:

Degenerative change involving the medial compartment of the knee with interval development of full-thickness cartilage denuding anteriorly within the medial compartment.

Mild degenerative change medially within the patellofemoral joint along the medial aspect of the trochlear surface...

Small joint effusion.
- The worker underwent a total knee replacement in October 2010 and the WSIB appears to have accepted entitlement and paid benefits for this surgical procedure. The pre- and post-operative diagnosis was “osteoarthritis right knee.”

[22] The WSIB concluded that the worker’s pre-existing condition was moderate. In reaching this conclusion, the ARO noted that the worker did not have any prior knee symptoms and she was capable of performing her strenuous pre-accident duties, which involved cleaning hospital rooms. Board policy does not provide detailed guidelines for determining the significance of a pre-existing condition, but I note that Board policy does not require a pre-accident disability in order to establish that a pre-existing condition was major, although that may often be the case.

[23] In this case, the medical documentation refers to extensive degeneration in the knee, including a degenerative tear. In some areas of the knee, the degeneration was described as “severe” and graded as Class III. The worker underwent arthroscopic surgery and went on to require a total knee replacement with a diagnosis of “osteoarthritis.” Although the worker did not have a pre-accident disability, the extent of the worker’s disability following the accident was much more severe and prolonged than would be expected from this type of injury. In particular, the worker underwent two surgeries, and the second surgery was based only on the diagnosis of osteoarthritis. I infer from the evidence that the pre-existing degeneration in the worker’s knee played a major role in prolonging the claim and the need for surgery.

[24] Since the accident history was moderate and the pre-existing condition was major, the employer is entitled to 75% cost relief.

DISPOSITION

[25] The appeal is allowed in part.

[26] The employer is entitled to 75% cost relief on the costs of this claim.

DATED: February 22, 2013

SIGNED: R. McCutcheon